

Pre-Operative Care of a Patient

Contents

Pre-Operative Care of a Patient (O&G)	1
Introduction	1
Classification of gynaecological operations	1
Pre-operative procedures	2
History taking	2
Physical examination	2
Investigations	3
Admission to hospital	3
Re-evaluation on admission	3
Informed consent	4
Operation list	4
Pre-anaesthetic visit	4
Patient preparation	4
Pre-operative prophylaxis	5

Pre-Operative Care of a Patient (O&G)

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Introduction

Pre-operative care encompasses **all the preparations made to ensure that a successful outcome is obtained in any surgery.**

They are measures directed at ensuring that the patient is in the **best possible condition physically and psychologically** for the surgery, and that post-operative recovery is satisfactory — thereby reducing the risks of both short and long term complications or morbidity.

Pre-operative care must therefore, of necessity, involve **the spectrum of medical care workers.**

Classification of gynaecological operations

Gynaecological operations may be classified into:

- Elective or Emergency
- Major or Minor
- Day case or in-patient
- Abdominal or Vaginal
- Diagnostic or Therapeutic

- Minimal Access Surgery

All of these have varying aspects of pre-operative care.

Pre-operative procedures

History taking

Vital and detailed. Should include:

- Presenting complaints
- Gynaecological and obstetric history
- Past medical and surgical history
- Past and present drug use
- Family and social history
- A review of systems

It begins in the outpatient department and is continued on admission, prior to surgery — especially for electives. For emergency operations, rapid preparations are carried out as the patient is admitted.

Physical examination

Should be thorough, paying attention to **extragenital systems**. Assessment of surgical and anaesthetic risks should be made.

History taking and physical examination are the **most important components of pre-operative assessment**, and dictate the need for other perioperative measures including laboratory tests.

General examination may reveal pallor (anaemia) and jaundice, which may point to liver disease.

Cardiovascular system — minimally check for:

- Pulse
- Blood pressure
- Heart sounds
- Presence or absence of dependent oedema

Tachycardia is usually the **first sign of cardiovascular compromise** and an important sign in ectopic pregnancy, for instance. Hypertension or hypotension would influence the assessment of anaesthetic or surgical risk. Heart sounds may provide evidence of cardiac disease.

Respiratory system — careful assessment may reveal respiratory infections, including tuberculosis, which may necessitate postponement of elective surgery or influence the choice of anaesthetic agent. Chronic diseases like asthma should also be considered when deciding on anaesthetics.

GIT — gynaecological symptoms may be due to GIT disorders; e.g. appendicitis may mimic ectopic pregnancy. Liver disease may affect metabolism of anaesthetic drugs.

Urinary system — exclude UTI. Bear in mind that obstructive uropathy may occur with pelvic tumours, e.g. fibroids, or as part of a malignant process.

Endocrine — diabetes mellitus is an important disorder that should be excluded, as should thyroid disease. These may be suggested by the patient's history and physical examination findings, and require further investigation.

Investigations

Necessary to confirm diagnosis. May be required to assess surgical and anaesthetic risk.

Actual investigations are **variable and individualised**, and dependent on the procedure.

Avoid a battery of investigations routinely, as this would increase cost.

Full blood count — haemoglobin concentration should be determined for **all patients**. A minimum of **10 g/dL** is traditionally accepted for elective procedures. However, in emergencies such as ruptured ectopic pregnancy, a low Hb should **not** preclude surgery, since the aim is prompt arrest of bleeding.

Platelet count — necessary in a missed abortion, or where there may be a problem with coagulation.

Hb genotype — necessary, as anaesthesia may trigger sickling crises.

Blood grouping and cross-matching — indicated where excessive blood loss is a distinct possibility. However, note that blood transfusion should not be undertaken lightly in the face of the AIDS pandemic and the risk of other transfusion-transmitted infections. Give consideration to alternatives to heterologous blood transfusion.

Ultrasound scan — may be useful to exclude pregnancy or obstructive uropathy.

Chest X-ray — should be done if there is hypertension, or known cardiac or respiratory problems. Routine if over **50 years**.

ECG — indicated in women over **60 years**, those who are hypertensive, or those with known cardiac disease.

Admission to hospital

Duration of admission prior to surgery depends on the type of surgery and the need for in-patient preparation.

It should not be unnecessarily long, because of psychological and economic resources, and so as not to deny bed space to patients who require hospitalisation.

Re-evaluation on admission

- Clerk again in detail
- Check investigations for completeness
- Check diagnosis and intended operation

Consider the following points:

- Any change in the patient's condition
- Any intercurrent illness
- Is the procedure still necessary?
- Is it the best for the patient?
- Any additional procedure to be done at the same time?

Consultation with other specialties should be guided by the patient's condition. **Medical consultation** is required for women with diabetes mellitus, hypertension or other medical disorders. **Pre-operative surgical consultation** should be considered if there is a possibility of bowel surgery or involvement of abdominal organs, as in known or suspected malignant disease or septic abortion.

Informed consent

Important, and should be obtained **without duress**.

- Involves a discussion with the patient in words that she can easily understand
- Explain the planned procedure, its indication and the alternatives
- Address the anticipated outcome and potential benefits/complications, ensuring that the patient is **not terrified into rejecting a beneficial operation**
- The consent form should be signed by the patient; but if a minor, consent should be obtained from parents/guardian
- Consent obtained should **not be restrictive**, but such that the surgeon is able to carry out additional procedures that may become necessary intra-operatively

Operation list

- Identifies the patients for surgery and the order in which the operations should be performed
- Should be produced in agreement with the anaesthetist and submitted to the theatre **well in advance**
- This enables theatre staff to make adequate arrangements for special equipment/materials if need be

Pre-anaesthetic visit

Done by the **anaesthetist, who is the final clinical gatekeeper**.

- Depends on the patient's condition and type of surgery
- Helps establish rapport and allay anxiety
- Prescribes the type of anaesthesia and premedication

NPO (nil per os) — traditionally 8 hours before anaesthesia, to ensure the stomach is empty and prevent regurgitation and aspiration. Achieved by overnight fast in elective cases. However, avoid prolonged fasting to prevent hypoglycaemia. Guidelines that will prevent pre-operative fasting beyond **6 hours** are being advocated.

Patient preparation

- Remove elaborate hairstyles and hairpieces
- Morning bath
- **Hair removal** should be limited to the area of incision if required. Risk of infection is reduced if this is done **shortly before surgery**, compared with the previous night
- **Nail polish** should be removed, as it may affect pulse oximeter readings
- **Lipstick** may affect assessment and should be washed off, especially for day cases
- **Bowel preparation** — not routinely done. Indicated when bowel surgery is anticipated
- **Bladder catheterisation**, if required, should be done in theatre (under anaesthesia)
- **Pre-operative cleansing of the vagina** — not routinely done, and has been shown not to be useful
- **Pre-medication** administered according to anaesthetic prescription

Patient preparation in the ward is usually done by the nursing staff, who must be **friendly disposed** to the patient. A good night's sleep and friendly faces make the patient relaxed and ready for surgery.

Pre-operative prophylaxis

Antibiotic prophylaxis — pre-operative antibiotic prophylaxis has now been shown to be **more cost effective than post-operative therapy for seven days**. Choice of antibiotics depends on local sensitivity patterns.

DVT prophylaxis — assess for risks of DVT and administer prophylaxis accordingly, e.g.:

- Adequate hydration
- Graduated compression stockings
- Heparin

Prolonged pre-operative bed rest may increase the risk of post-operative thromboembolism.